

Zepbound — brand tirzepatide

Weight Loss & Metabolic Health · Provider Reference

KORB Health Group · korb-glpl-data.js v2.14 · built 2026-09-06

Everything needed to prescribe Zepbound brand tirzepatide, complete on its own. Values are copied literally into Tebra — do not paraphrase, and do not adjust quantity, refill or days supply.

At a glance

Type	Brand product — dispensed by the manufacturer, not compounded by KORB
Fulfilment	Every brand-name pathway — LillyDirect, NovoCare and the patient's local pharmacy. One fee, one code, no variation by product or channel.
Order via	Tebra standard prescription
Visit fee	\$79 · FITGLP1001
Medication pricing	Not held by KORB — see below

Brand product

Brand product dispensed through the manufacturer programme. Not compounded. Fulfilment, payment and shipping are handled by the manufacturer, not KORB.

Program options

Zepbound (tirzepatide) — brand name

Program	Quantity	Refill	Days supply	When to use
4-week (28-day)	1 pen	0	28	New prescription or any dose change. Use until the dose is stable.
8-week (56-day) — 1 pen + 1 refill	1 pen	1	28	Clinically stable patients only. Each fill is one pen covering 4 weeks.

12-week (84-day) — 1 pen + 2 refills	1 pen	2	28	Clinically stable patients on a settled dose. Three fills of one pen, four weeks each.
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Why the longer injectable programs carry refills

Brand pharmacies will not ship more than a 4-week supply of an injectable at a time. The 8-week and 12-week programs are therefore the SAME 4-week fill with refills, not a larger shipment - which is why days supply stays 28 on all three. It describes the fill, not the program.

KORB does not control brand fulfilment

KORB does not control brand fulfilment. The prescription is sent like any other, and the patient pays the pharmacy directly. Price and coverage are between the patient and the manufacturer programme.

Tebra — copy these values literally

Do not paraphrase, and do not adjust quantity, refill or days supply.

Zepbound (tirzepatide) — brand name

Tebra Standard prescription

2.5 mg — 4-week

Drug	Zepbound kwikpen 2.5 mg/0.6 ml(10 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 2.5 mg SQ 4-week
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	Inject 2.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.

2.5 mg — 8-week

Drug	Zepbound kwikpen 2.5 mg/0.6 ml (10 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 2.5 mg SQ 8-week
Quantity	1
Refill	1
Days Supply	28
Patient Instructions	Inject 2.5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

2.5 mg — 12-week

Drug	Zepbound kwikpen 2.5 mg/0.6 ml (10 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 2.5 mg SQ 12-week
Quantity	1
Refill	2
Days Supply	28
Patient Instructions	Inject 2.5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

5 mg — 4-week

Drug	Zepbound kwikpen 5 mg/0.6 ml (20 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 5 mg SQ 4-week
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	Inject 5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.

5 mg — 8-week

Drug	Zepbound kwikpen 5 mg/0.6 ml (20 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 5 mg SQ 8-week
Quantity	1
Refill	1
Days Supply	28
Patient Instructions	Inject 5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

5 mg — 12-week

Drug	Zepbound kwikpen 5 mg/0.6 ml (20 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 5 mg SQ 12-week
Quantity	1
Refill	2
Days Supply	28
Patient Instructions	Inject 5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

7.5 mg — 4-week

Drug	Zepbound kwikpen 7.5 mg/0.6 ml (30 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 7.5 mg SQ 4-week
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	Inject 7.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.

7.5 mg — 8-week

Drug	Zepbound kwikpen 7.5 mg/0.6 ml(30 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 7.5 mg SQ 8-week
Quantity	1
Refill	1
Days Supply	28
Patient Instructions	Inject 7.5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

7.5 mg — 12-week

Drug	Zepbound kwikpen 7.5 mg/0.6 ml(30 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 7.5 mg SQ 12-week
Quantity	1
Refill	2
Days Supply	28
Patient Instructions	Inject 7.5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

10 mg — 4-week

Drug	Zepbound kwikpen 10 mg/0.6 ml (40 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 10 mg SQ 4-week
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	Inject 10 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.

10 mg — 8-week

Drug	Zepbound kwikpen 10 mg/0.6 mL (40 mg/2.4 mL) subcutaneous pen injector
Name	Zepbound 10 mg SQ 8-week
Quantity	1
Refill	1
Days Supply	28
Patient Instructions	Inject 10 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

10 mg — 12-week

Drug	Zepbound kwikpen 10 mg/0.6 mL (40 mg/2.4 mL) subcutaneous pen injector
Name	Zepbound 10 mg SQ 12-week
Quantity	1
Refill	2
Days Supply	28
Patient Instructions	Inject 10 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

12.5 mg — 4-week

Drug	Zepbound kwikpen 12.5 mg/0.6 mL (50 mg/2.4 mL) subcutaneous pen injector
Name	Zepbound 12.5 mg SQ 4-week
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	Inject 12.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.

12.5 mg — 8-week

Drug	Zepbound kwikpen 12.5 mg/0.6 mL (50 mg/2.4 mL) subcutaneous pen injector
Name	Zepbound 12.5 mg SQ 8-week
Quantity	1
Refill	1
Days Supply	28
Patient Instructions	Inject 12.5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

12.5 mg — 12-week

Drug	Zepbound kwikpen 12.5 mg/0.6 mL (50 mg/2.4 mL) subcutaneous pen injector
Name	Zepbound 12.5 mg SQ 12-week
Quantity	1
Refill	2
Days Supply	28
Patient Instructions	Inject 12.5 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

15 mg — 4-week

Drug	Zepbound kwikpen 15 mg/0.6 mL (60 mg/2.4 mL) subcutaneous pen injector
Name	Zepbound 15 mg SQ 4-week
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	Inject 15 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.

15 mg — 8-week

Drug	Zepbound kwikpen 15 mg/0.6 ml (60 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 15 mg SQ 8-week
Quantity	1
Refill	1
Days Supply	28
Patient Instructions	Inject 15 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

15 mg — 12-week

Drug	Zepbound kwikpen 15 mg/0.6 ml (60 mg/2.4 ml) subcutaneous pen injector
Name	Zepbound 15 mg SQ 12-week
Quantity	1
Refill	2
Days Supply	28
Patient Instructions	Inject 15 mg subcutaneously once weekly. Multi-dose pen – discard after 4 doses.

Pricing and charge codes

Zepbound (tirzepatide) — brand name — 4-week

Program	Price	Charge code
Prescription visit fee	\$79	FITGLP1001

Manufacturer pricing changes without notice and KORB does not control it. Storing it creates a maintenance burden and risks providers quoting incorrect pricing to patients. Price and coverage are the patient's responsibility.

Zepbound (tirzepatide) — brand name — 8-week

Program	Price	Charge code
Prescription visit fee	\$79	FITGLP1001

Manufacturer pricing changes without notice and KORB does not control it. Storing it creates a maintenance burden and risks providers quoting incorrect pricing to patients. Price and coverage are the patient's responsibility.

Includes: Telehealth visit, medication, shipping and supplies. Never quote pricing to a patient without confirming with Operations.

Clinical reference — Tirzepatide — dual GIP and GLP-1 receptor agonist

Indications

- Chronic weight management in adults with obesity (BMI ≥ 30 kg/m²), or overweight (BMI ≥ 27 kg/m²) with at least one weight-related comorbid condition
- Long-term maintenance of weight reduction
- Improvement of glycemic control in patients with type 2 diabetes mellitus
- Treatment of moderate-to-severe obstructive sleep apnea in adults with obesity

What KORB treats

KORB treats weight loss only. Every other indication above stays with the patient's PCP or specialist - KORB does not manage type 2 diabetes or obstructive sleep apnea, and does not order or monitor labs for them. A patient carrying one of those diagnoses is acceptable on the weight-loss program provided that care continues elsewhere.

Identifying the appropriate candidate

Age. Adults 18 years and older. KORB does not treat patients under 18.

BMI

- BMI greater than 25 is overweight
- BMI greater than 30 is obese

Diabetes Note. KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Definition

A 39-amino-acid synthetic peptide based on the native GIP sequence, engineered to agonise both the glucose-dependent insulinotropic polypeptide (GIP) receptor and the GLP-1 receptor. A C20 fatty diacid moiety extends the half-life to roughly five days, allowing once-weekly dosing. FDA-approved as Mounjaro for type 2 diabetes and as Zepbound for chronic weight management and obstructive sleep apnoea in adults with obesity.

Mechanism

- Dual incretin agonism — both the GIP and the GLP-1 receptor.
- GLP-1 component: glucose-dependent insulin secretion, glucagon suppression, delayed gastric emptying, central appetite reduction.
- GIP component: enhanced insulin secretion and improved insulin sensitivity in adipose tissue.
- GIP agonism may also blunt the nausea seen with GLP-1 activity alone, one proposed reason tolerability holds up at higher relative efficacy.

Clinical evidence

- SURMOUNT-1: mean weight loss around 20% of body weight at 15 mg over 72 weeks — the largest effect seen with a pharmacological agent in this class.
- SURPASS programme: glycaemic efficacy in type 2 diabetes, generally exceeding semaglutide in head-to-head comparison.
- SURMOUNT-OSA: supported the obstructive sleep apnoea indication.
- As with semaglutide, weight regain after discontinuation is well documented.

Absolute contraindications

- Personal or family history of medullary thyroid carcinoma (MTC)
- Multiple endocrine neoplasia syndrome type 2 (MEN2)
- Known hypersensitivity to tirzepatide or any component of the formulation
- Pregnancy, breastfeeding, or planning pregnancy

Cautions

- History of pancreatitis — evaluate carefully before initiating
- Gastroparesis or other significant gastrointestinal motility disorder
- Active gallbladder disease or prior gallbladder-related surgical complications
- Severe renal impairment (eGFR below 30 mL/min/1.73 m²)
- Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
- History of suicidal ideation or active depression — monitor and ask

Medication interactions

- ORAL CONTRACEPTIVES: tirzepatide reduces the effectiveness of oral hormonal contraception. Advise a non-oral method, or add a barrier method, for four weeks after initiation and for four weeks after each dose increase. This is specific to tirzepatide and is easy to miss.
- Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
- Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
- Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
- ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. HOLD for 7 days (one week) before an elective procedure requiring sedation, or for whatever longer period the anaesthesia team or the surgeon's office requires - their instruction takes precedence over this one. Tell the patient to disclose GLP-1 use to any surgeon, proceduralist or anaesthetist.
- Other GLP-1 or dual incretin agonists: do not combine.

Side effects

COMMON

- Pain, redness or swelling at the injection site

- Nausea and vomiting
- Diarrhea
- Stomach pain
- Constipation
- Low appetite
- Headaches
- Dizziness

LESS COMMON

- Pancreatitis
- Gastroparesis
- Bowel obstruction
- Gallstone attacks

Monitoring

- Weight and BMI at each visit
- Ask about blood pressure — monitored by the PCP, not measured by KORB
- Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB
- Gastrointestinal tolerance, especially through dose escalation
- Abdominal pain that is severe or radiates to the back — assess for pancreatitis
- Right-upper-quadrant pain — assess for gallbladder disease
- Contraceptive method if the patient uses oral hormonal contraception
- Mood and mental health
- Lean mass preservation — protein intake and resistance training
- Injection site and adherence

Patient counseling

Tirzepatide works on two gut hormone pathways rather than one, which is why it tends to produce more weight loss than semaglutide. It slows stomach emptying and reduces appetite. Nausea is the most common side effect and is usually worst in the days after a dose increase. This is a compounded preparation rather than the brand-name product. If you take a birth control pill, it may not work as reliably for four weeks after we start and after each dose increase, so use a backup method. Weight tends to come back if the medication stops. Tell any surgeon or anaesthetist that you are taking this.

Escalation and discontinuation

HARD STOP

- Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate.
- Hypersensitivity or anaphylaxis-type reaction.

- Pregnancy identified or confirmed.
- Medullary thyroid carcinoma or MEN2 identified at any point.
- Persistent severe vomiting with dehydration or acute kidney injury.

FLAG AND REASSESS

- Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward.
- Right-upper-quadrant pain, or gallstone symptoms.
- Rapid or excessive weight loss, or loss of lean mass.
- New or worsening depression, or any mention of suicidal thoughts.
- Worsening retinopathy in a diabetic patient during rapid glycaemic improvement.
- Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely.
- Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

Chart attestation

Patient counseled on tirzepatide as a dual GIP/GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, reduced oral contraceptive effectiveness with the recommended backup precautions, the need to disclose use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

Note. DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

PRIMARY

- E66.9 — Obesity, unspecified
- E66.01 — Morbid (severe) obesity due to excess calories
- E66.3 — Overweight

SECONDARY

- Z68.— — Body mass index (add the matching BMI code)
- G47.33 — Obstructive sleep apnea (if documented and relevant)
- Z71.3 — Dietary counseling and surveillance
- Z72.3 — Lack of physical exercise

Clinically reviewed

Reviewed and signed off by Donald Stevenson, PA-C (Director of Clinical Operations and Lead Provider) on 2026-09-06, against korb-glp1-data.js v2.13.

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Questions and escalation

- Pharmacy or shipping — Operations
- Charge codes and billing — Operations
- Clinical protocol — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute an unavailable pharmacy
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without Operations

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